WRITING SAMPLE · POLICY ANALYSIS & PROGRAM EVALUATION

The Impact of Parental Substance Use on Child Well-Being

A thematic literature review pairing national and state evidence with agency administrative data on caregiver substance use, prenatal exposure and service utilization.

DOCUMENT	**Thematic literature review**
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Introduction

Parental substance use and parental substance use disorder have been identified as significant factors influencing various aspects of a child's life, from immediate safety concerns to long-term outcomes. This memorandum explores the multifaceted impact of parental substance use on children's short- and long-term well-being, pairing the published evidence base with the agency's own aggregate administrative data.

The Office of Human Services Policy found that in an average year from 2015 to 2019, more than 21 million children in the United States lived with a parent who misused substances, and more than 2 million children lived with a parent who had a substance use disorder. Children younger than 12 had a higher likelihood of living with parents with a substance use disorder than children aged 12 and older.[1]

The 2022 National Survey on Drug Use and Health estimates the amount of general substance use in the month prior to interview among people aged 12 and over. The substance with the greatest number of past-month users is alcohol; of the 137.4 million alcohol users, 61.2 million are binge alcohol users (44.5% of alcohol users) and 15.1 million were considered heavy alcohol users (26.3% of binge alcohol users and 11.7% of alcohol users).[2]

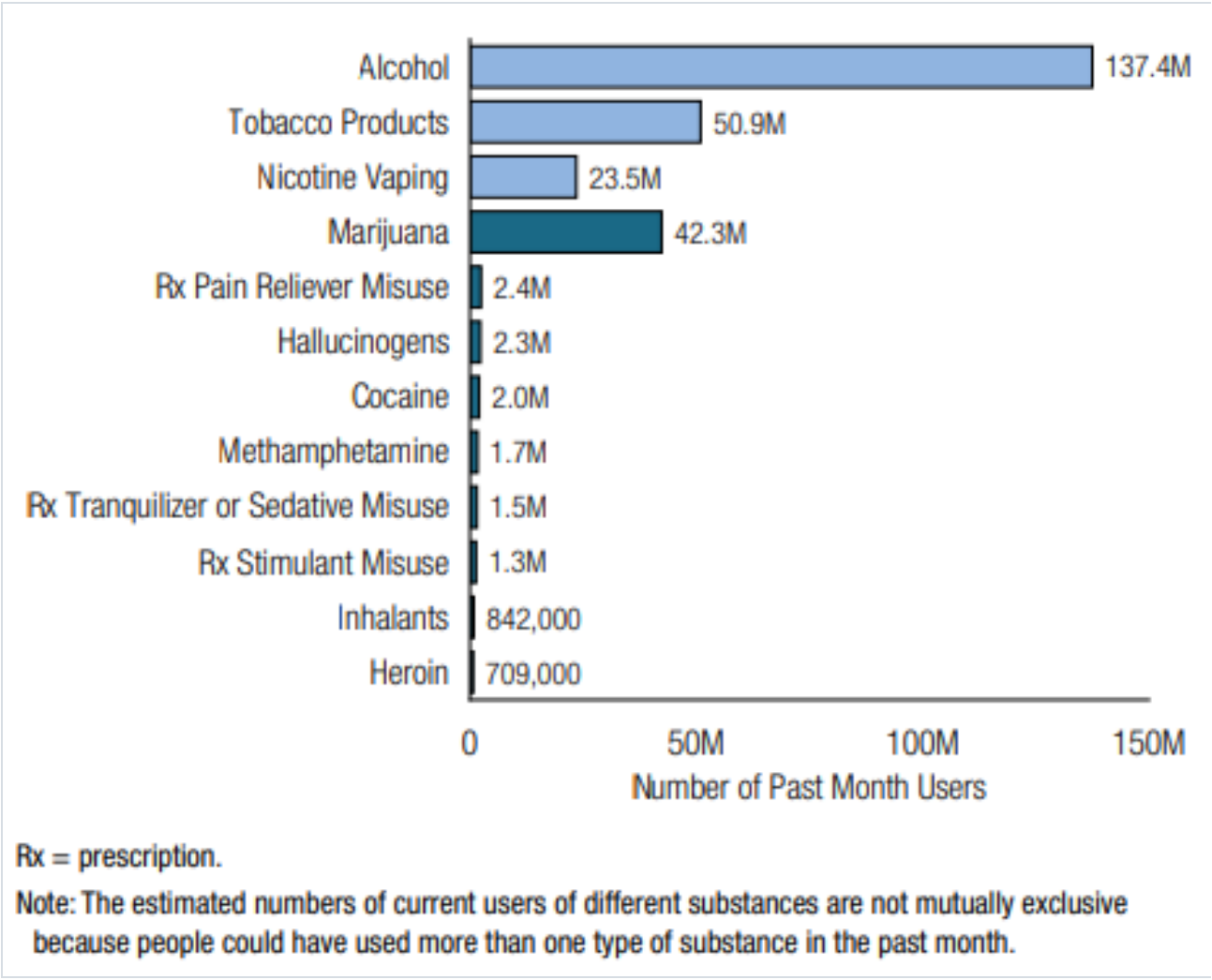


Figure 1. Past-month substance use among people aged 12 or older, 2022. Adapted from the 2022 National Survey on Drug Use and Health.[3]

In Indiana, it was reported in 2021 that 70% of adults aged 35 to 44 reported current dual use of marijuana and cigarettes. Marijuana use is higher in younger age groups, among those with lower annual incomes, and among those with less educational attainment. Of Indiana adults with an annual income of $15,000 or less, 13.1% reported current marijuana use, and 13.2% of adults who had not earned a high school diploma reported current marijuana use.[4]

In the 2022 Indiana Youth Survey summary report, parents were the primary source of alcohol for youth in 7th and 8th grades, while older students were most likely to report obtaining alcohol at a party. Approximately 20.1% to 22.6% of youth reported that other family members gave alcohol to them.[5]

One case-control study examined the treatment gap among Medicaid-covered caregivers requiring mental health and substance use disorder treatment. The study found that despite the availability of effective treatment options covered by Medicaid, a significant number of these caregivers are not receiving necessary treatment, even when their

children have been referred to child welfare services. This research underscores the need for further investigation into the barriers to treatment access and utilization among this population, with the aim of developing effective strategies to address these challenges. The findings have significant implications for healthcare policy and practice, particularly in improving the well-being of caregivers and the safety of their children.

Alcohol use in the United States has increased since the pandemic

According to the National Institute on Alcohol Abuse and Alcoholism Surveillance Report #120, using alcoholic beverage sales data, per capita consumption of ethanol from all alcoholic beverages combined in 2021 was 2.51 gallons — a 2.9% increase from 2.44 gallons in 2020 and a 5.5% increase from 2.38 gallons in 2019, the largest two-year increase since 1967–1969.[6]

One study examines trends toward increased alcohol consumption and increased use of other substances during the pandemic. Researchers found that 2020 was associated with a downward deviation in 30-day and binge drinking prevalence, an upward deviation in daily drinking prevalence, and among drinkers an upward deviation in the frequency of 30-day and binge drinking, along with changes in drinking contexts and reasons.[7]

Immediate safety impacts

Prenatal substance exposure

According to self-reported data from 2019, about 7% of women reported using prescription opioid pain relievers during pregnancy.[8]

Methamphetamine, the second most widely used illegal drug globally after cannabis, has limited data available on its effects on pregnant users. However, emerging evidence suggests that these women are more vulnerable, marginalized and impoverished than mothers using other drugs.

Pregnancy outcomes tend to be worse for methamphetamine users, with a higher prevalence of concurrent health and psychosocial issues. Newborns exposed to methamphetamine have a higher risk of complications around the time of birth, exhibit different characteristics at birth compared with infants exposed to dependency drugs such as opioids, and struggle more with neurological adaptation and feeding.

Limited studies, including neuroimaging and cohort studies, suggest that children exposed to methamphetamine may continue to experience neurocognitive deficits into adulthood. Current clinical guidelines for treating pregnant women exposed to substances focus primarily on opioids, with little consideration given to the effects of prenatal methamphetamine exposure.[9]

Long-term consequences

Not in education, employment or training

One study found that 4.4% of individuals in Sweden had been exposed to parental substance use disorder before 17 years of age, and that this exposure is associated with increased risk of not being in education, employment or training during early adulthood. The risks found were highest at age 17 to 19 for both males and females, with the risk decreasing with greater age.[10]

Problematic parental substance use, family structure and their outcomes are explored in another study. Problematic parental substance use and other family problems often co-occur. The study found that children who live with a parent struggling with substance abuse do not necessarily have worse outcomes if they live with that parent for a longer time. The group that faced the most challenges were those who lived with a parent with substance abuse issues for a short time and did not live with both parents. These young people were more likely to be unemployed, have mental health issues, and face legal trouble.

The study also pointed out that the measure used only shows how long the child lived with the parent, not how long they were exposed to substance abuse. One reason for the higher risk in the group with brief exposure could be that their parents separated early; some research suggests that children who experience parental divorce early in life face more challenges later. Another possible reason could be high levels of family dysfunction and severe substance use, which were not factors included in this study. These issues might lead to parents separating soon after the child's birth. The overall family environment during childhood might therefore be more important than whether the child lives with the parent with substance abuse issues. This is supported by other studies showing that parental problems and family functioning can affect children's behavior over time. The study also suggested that instability in the family structure, rather than the structure itself, might be linked to negative outcomes.[11]

Educational attainment

One study identified four categories of young individuals, each experiencing different degrees of family-related issues. The findings highlight that young individuals who encounter parental substance use disorder, either as the main family-related issue or in conjunction with other family-related problems, are more likely to face adverse academic outcomes. This research found that parental substance use disorder significantly influences the academic performance of young individuals and their likelihood of pursuing further education after compulsory schooling.

The findings are in line with previous research suggesting that parental substance use disorder has independent effects on the academic outcomes of children. Despite the

presence of other family-related issues, it stands out as a crucial factor affecting the well-being and academic success of young people.[12]

Economic status

Parental substance use is a complex issue with far-reaching implications, not only for the parent struggling with addiction but also for their children. The economic status of both the parent and the child can be significantly affected. When a parent struggles with substance abuse, it often leads to financial instability due to factors such as job loss, medical costs and legal issues. This instability can create a challenging environment for children, affecting their development and future economic prospects. As these children grow up, they may face obstacles in education and employment, potentially leading to a cycle of economic disadvantage.

Child welfare involvement

Each year approximately 335,000 children are reported to the child welfare system due to parental substance use being identified as a risk factor. Among these reported cases, about 136,000 children are confirmed victims of maltreatment, highlighting the direct correlation between parental substance use and child abuse or neglect. Around 95,000 children with a parent struggling with substance use are placed in out-of-home care, further compounding the trauma and instability these children experience. These figures underline the urgent need to address parental substance use and provide adequate support and intervention for the affected families.[13]

The literature clearly indicates that children face challenges when their parents, even if not dependent, misuse substances. This issue becomes more severe when both parents misuse alcohol and/or drugs, or when one or more parents misuse a mix of these substances.

Maternal alcohol misuse is particularly identified as a significant factor for poorer child health. This could be partly because mothers usually have a larger role in the child's early years. However, there is a lack of research on substances other than alcohol and on the impact of fathers' substance misuse, which could lead to an undue emphasis on mothers.[14]

Termination of parental rights

In some cases of parental substance abuse, where the substance use disorder compromises parenting ability, parental rights must be terminated due to physical and mental impairments, domestic violence, frequent arrests, incarceration and court involvement. One study found that parents with problematic substance use whose children are older are less likely than those with younger children to experience termination of parental rights. The findings also suggest that boys are significantly more

likely to be abused in these situations than girls, and that boys are significantly less likely than girls to be removed from their home. The study also found that mothers with problematic substance use of Black children were no more likely than those of white children to experience termination of parental rights, suggesting that these decisions are largely based on lack of treatment progress regardless of race or ethnicity.[15]

Health outcomes

One study claims that there are shared genetic factors between parental substance use disorder and offspring cognitive function, which suggests that cognitive deficits may constitute a genetically transmitted risk factor in substance use disorder.[16]

Another study found that hospitalizations of children for all reasons were more common if either parent had a substance abuse problem. The mother's substance abuse was linked to an increased risk of hospitalization for physical illness and psychiatric disorders, while the father's substance abuse was associated with an increased risk of hospitalization due to psychiatric disorders. The risks escalated further if both parents were substance abusers. The study concluded that parents' substance abuse can lead to various harms to children, potentially due to an unsafe environment, chronic stress and inadequate response to the child's needs. Collaborative work involving multiple professionals with substance-abusing parents and their children is therefore vital to mitigate the children's health risks.[17]

Research also indicates a strong link between parental substance misuse and a heightened risk of injuries, physical illnesses and mental health issues in young children. This elevated risk of injury suggests neglect, maltreatment, and in some instances even violence.

The increased prevalence of physical and mental health disorders in young children whose parents misuse substances can be attributed to various factors, including a poor living environment, inadequate nutrition and hygiene, poverty, social disadvantage, exposure to conflict and violence, and child abuse. In addition to the immediate negative impact on the child's safety and health due to lack of proper care and supervision, these conditions create chronic stress, which further compromises the child's health.

Regarding mental health conditions, the findings suggest that a child's separation from both parents, even if they are struggling with substance abuse, tends to heighten the child's susceptibility to mental disorders rather than diminish it.[18]

Potential mitigating factors

Employment status

In one study, researchers found that the potential for mothers to maltreat their children may be reduced through gainful employment that is personally satisfying. Job satisfaction was the only employment factor that showed an inverse correlation with substance use, as determined by biological testing and self-reports from participants.[19]

Tobacco, alcohol, prescription medication and other substance use screening

Literature indicates that a variety of interconnected behaviors, including excessive alcohol consumption, illegal drug use, substance use disorder, and recent history of substance use disorder within the past four years, play a significant role in explaining variation in child abuse rates. Parents who engage in hazardous substance use could potentially benefit from preventative measures or short-term interventions targeting both their substance use and their parenting practices. Administrative structures might also find it advantageous to monitor substance use patterns carefully for the purposes of assessing and designing future programs.[20]

One way to monitor potential problem substance use in parents would be the Tobacco, Alcohol, Prescription medication and other Substance use (TAPS) tool. The TAPS tool consists of a combined screening component and, for those who screen positive, a brief follow-up assessment. This tool has been shown to detect clinically relevant problem substance use. While it detects tobacco, alcohol and marijuana use disorders, the tool needs further refinement before it can be broadly recommended as a screener for substance use disorder generally.[21]

Parents under Pressure program

In a UK-based family-focused program, the study found that certain factors were linked to a decrease in child abuse potential after six months. The age of the child at the start of the program was a significant factor, with older children associated with a greater decrease in child abuse potential, especially in the Parents under Pressure program compared with usual treatment. Parents who initially had difficulty managing their emotions, or who were at higher risk due to their family situation, also saw a greater decrease in child abuse potential in both groups. Improvements in parental emotional regulation after treatment were linked to a greater decrease in child abuse potential, suggesting a mediating role for emotional regulation. Notably, the Parents under Pressure program was found to be more effective in improving parental emotional regulation than usual treatment.[22]

Internal agency statistics

The figures in this section are aggregate, statewide counts drawn from the agency's administrative data. No case-level records, county-level breakdowns or individual identifiers are reproduced.

Self-reported drug use by caregivers

There are several ways to assess caregiver substance use. Figure 2 illustrates caregiver risk assessments related to self-reported drug and alcohol use over a 12-month lookback, spanning January 2019 to December 2023. Notable trends include the following.

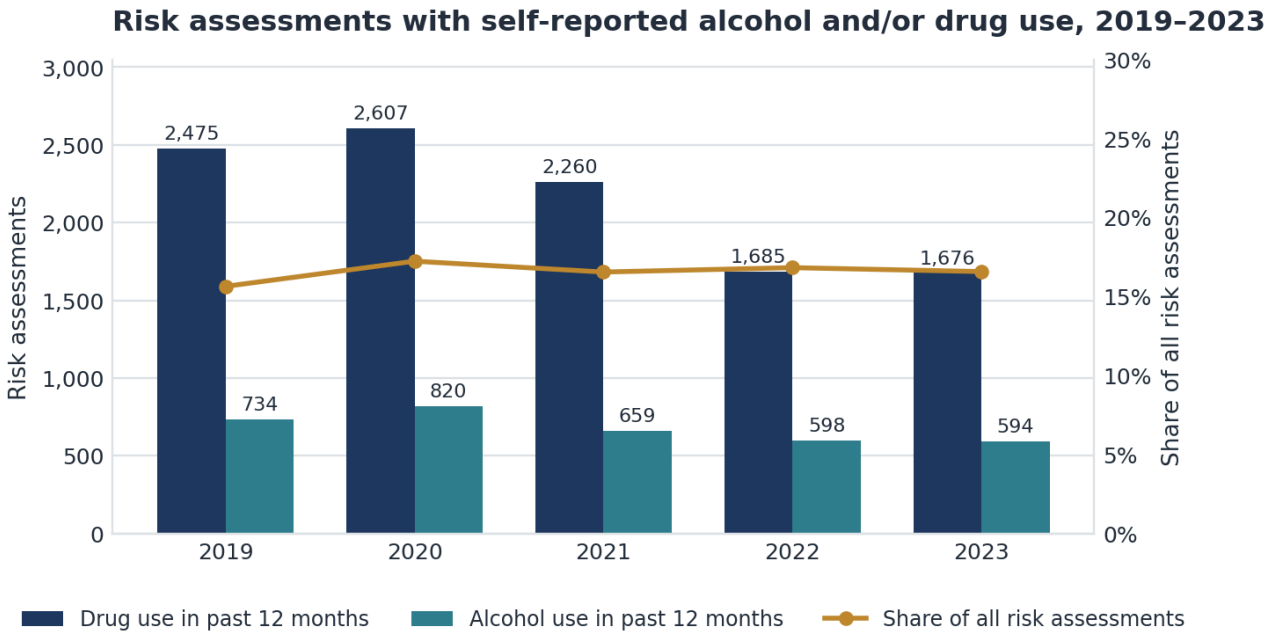


Figure 2. Risk assessments with self-reported alcohol and/or drug use, 2019–2023.

Percentage trends

- In 2019, the lowest percentage of risk assessments with self-reported alcohol and/or drug problems was approximately 15.64%.
- The highest percentage occurred in 2020, reaching around 17.23%.
- Since 2021 the rate has remained relatively stable at approximately 16.55%.

Drug use trends

- Instances of self-reported drug use within the past 12 months have decreased significantly.
- The peak was in 2020, with 2,607 assessments; by 2023 this had declined to 1,676 assessments.

Alcohol use trends

- Self-reported alcohol use has also decreased.
- In 2020 there were 820 assessments indicating alcohol use within the past 12 months; in 2023 this number had decreased to 594 assessments.

Caregiver risk assessments completed by case managers

Figures 3 and 4 depict caregivers' substance use needs as assessed by case managers. The number of caregiver risk assessments conducted has decreased significantly, from 3,847 assessments in 2019 to 1,713 assessments in 2023. Despite this decline, the proportions of risk assessment results remain consistent. There is a slight increase in the proportion of assessments indicating that caregivers have no substance use needs, suggesting a positive trend.

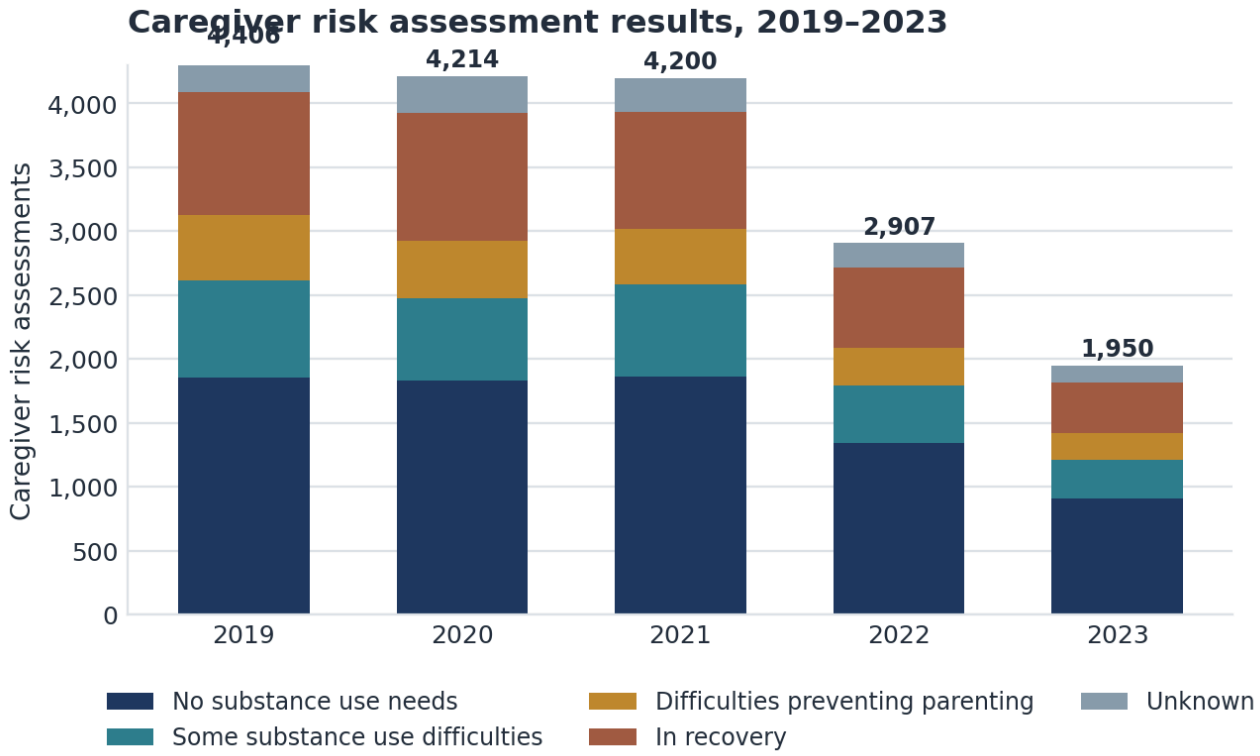


Figure 3. Caregiver risk assessment results, 2019–2023.

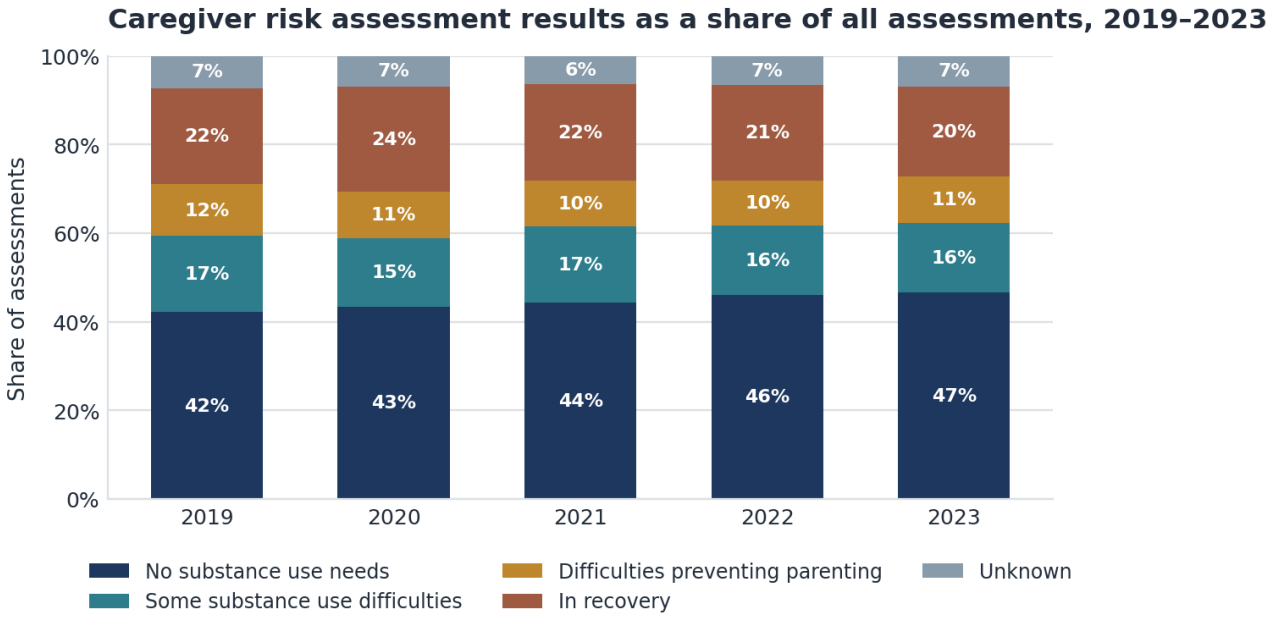


Figure 4. Caregiver risk assessment results as a share of all assessments, 2019–2023.

Allegations of drug-exposed infants

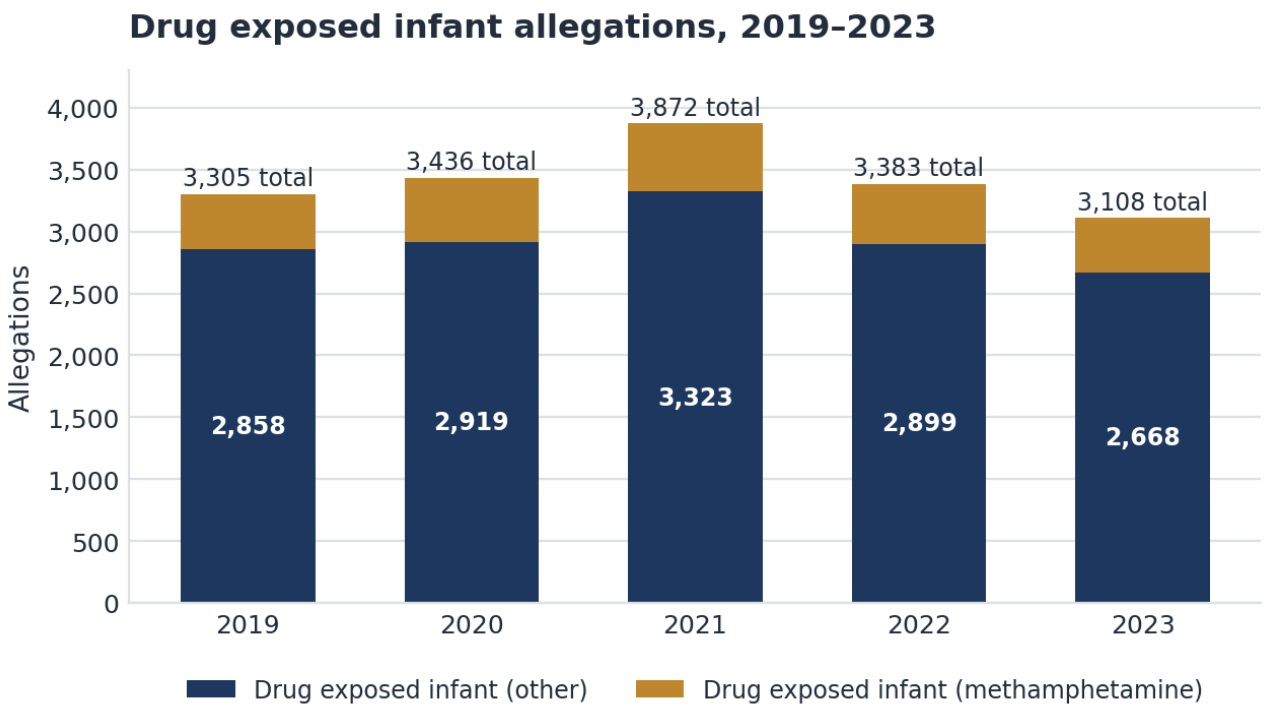


Figure 5. Drug exposed infant allegations, 2019–2023.

Drug exposure during pregnancy can have significant consequences for both infants and mothers. The key findings are as follows.

Methamphetamine exposure

- In 2019 there were 447 cases of methamphetamine-exposed infants.
- By 2021 this number had increased to 549, indicating a concerning trend.
- There was a slight decline in 2023, with 440 cases.

Other drug exposures

- In 2019 there were 2,858 cases of infants exposed to other drugs.
- The count remained stable in 2020 (2,919 cases) but increased significantly in 2021 (3,323 cases).
- By 2023 the number had decreased to 2,668.

Combined total

- The total number of drug-exposed infants, including both methamphetamine and other drugs, was 3,190 in 2019.
- It rose to 3,694 in 2021 but reversed in 2023, with a total of 2,971 cases.

Implications and interventions. Efforts are needed to address the rising trend of methamphetamine exposure among infants. However, the decline in 2023 suggests that interventions may be having an impact.

Substance use treatment referrals

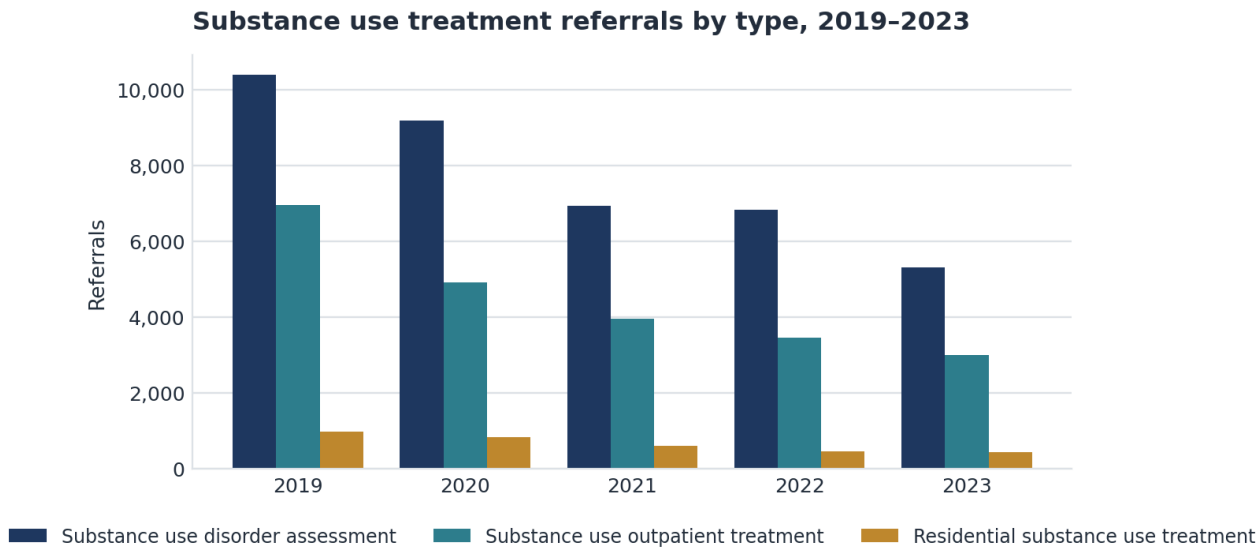


Figure 6. Substance use treatment referrals by type, 2019–2023.

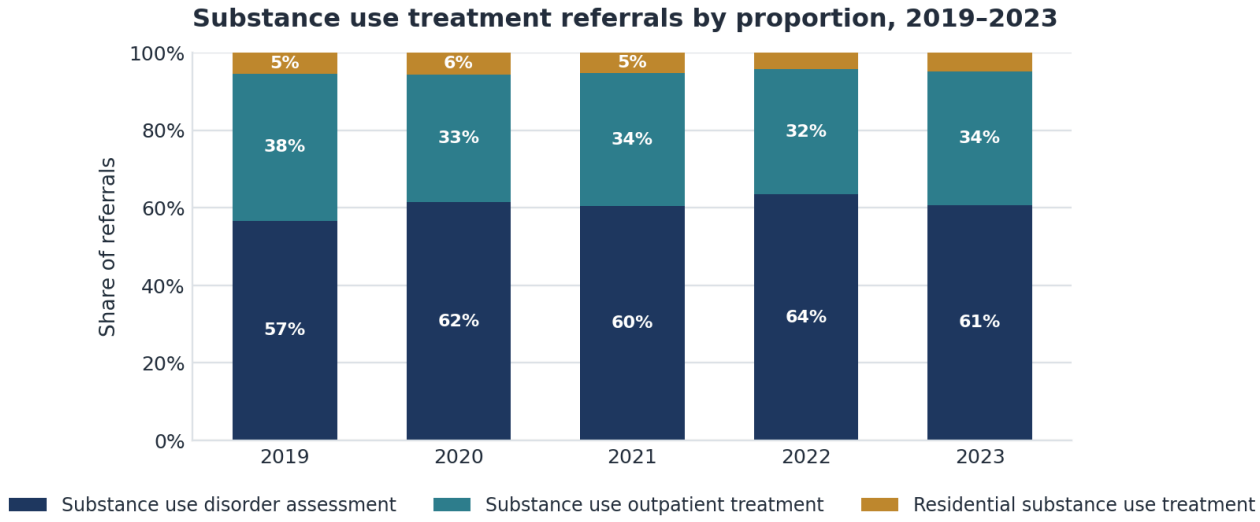


Figure 7. Substance use treatment referrals by proportion, 2019–2023.

Safe Systems reviews

Fatality reviews are conducted by the Safe Systems team if the agency, or a primary prevention provider such as Healthy Families Indiana or Nurse–Family Partnership, had history with the child or family in the preceding six months. Figure 8 shows the most

prevalent risk factors in families eligible for Safe Systems reviews in 2023; many families experience multiple co-occurring risk factors.

Regarding fatalities in 2023, 98 cases qualified for Safe Systems reviews, consisting of 61 fatalities and 27 near fatalities. THC was the most prevalent substance to which infants were exposed, representing 84% of the 43 reviewed infant fatalities involving substance exposure.[23]

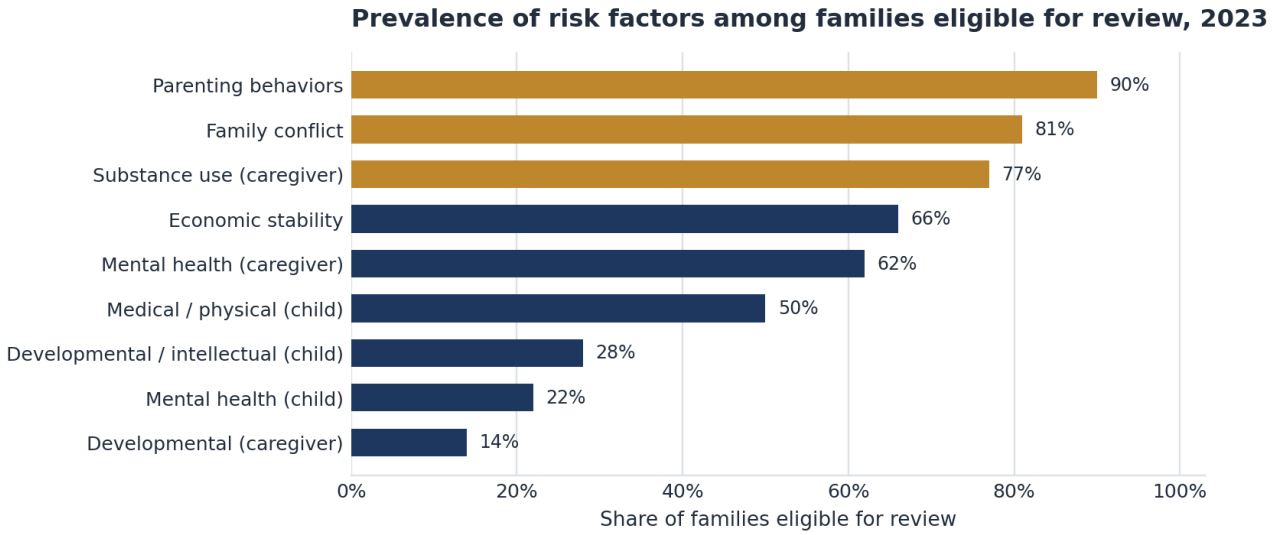


Figure 8. Prevalence of risk factors among families eligible for Safe Systems review, 2023. Adapted from the 2023 Safe Systems review findings.[24]

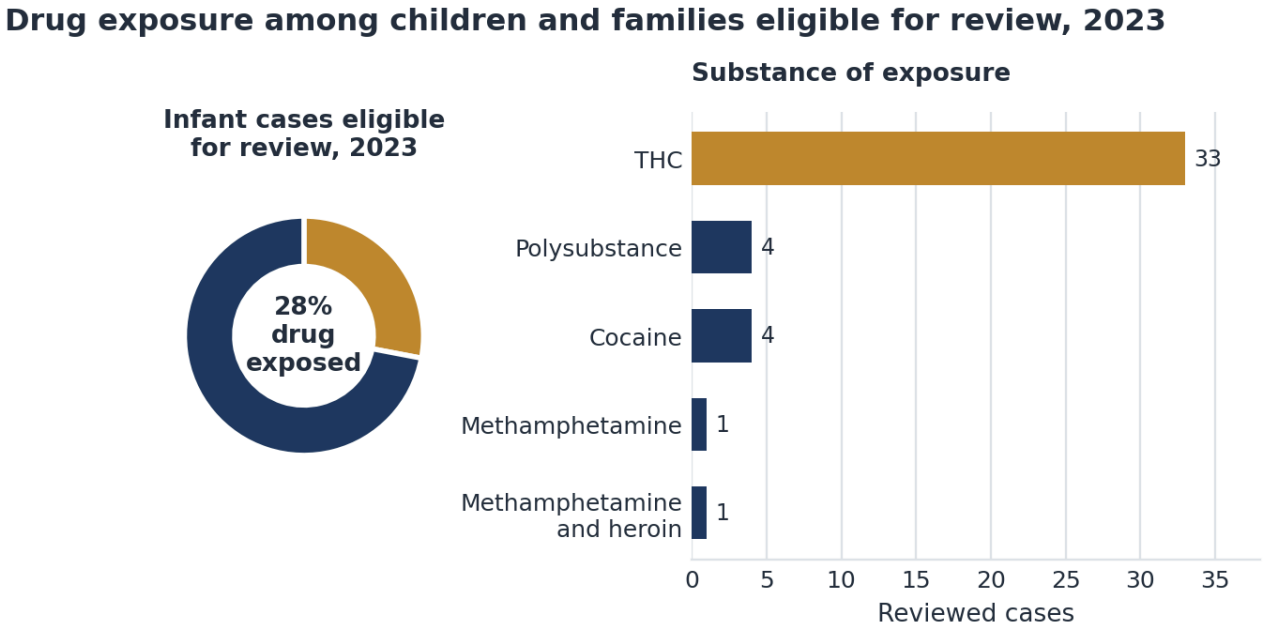


Figure 9. Drug exposure among children and families eligible for Safe Systems review, 2023.[24]

As of April 2024, Safe Systems had conducted a total of 29 fatality reviews. Thirty-seven percent of the critical incidents reviewed had a prior call to the Safe Assessment Closure Team, suggesting that there were warning signs or interactions before these incidents escalated. Twenty percent of the fatalities qualified for review; these cases likely involved significant factors or complexities that warranted further examination. Twenty-seven percent of the assigned reviews focused on drug-exposed infants, and the two most common substances these infants were exposed to were THC and methamphetamine. Fifty-five percent of cases had five co-existing risk factors: family conflict, mental health, substance use, economic stability and parenting behavior.

From January to April 2024, nine cases were eligible for Safe Systems review regarding infants exposed to drugs at birth. These infants were exposed principally to THC, as well as to methamphetamine and a combination of both substances.

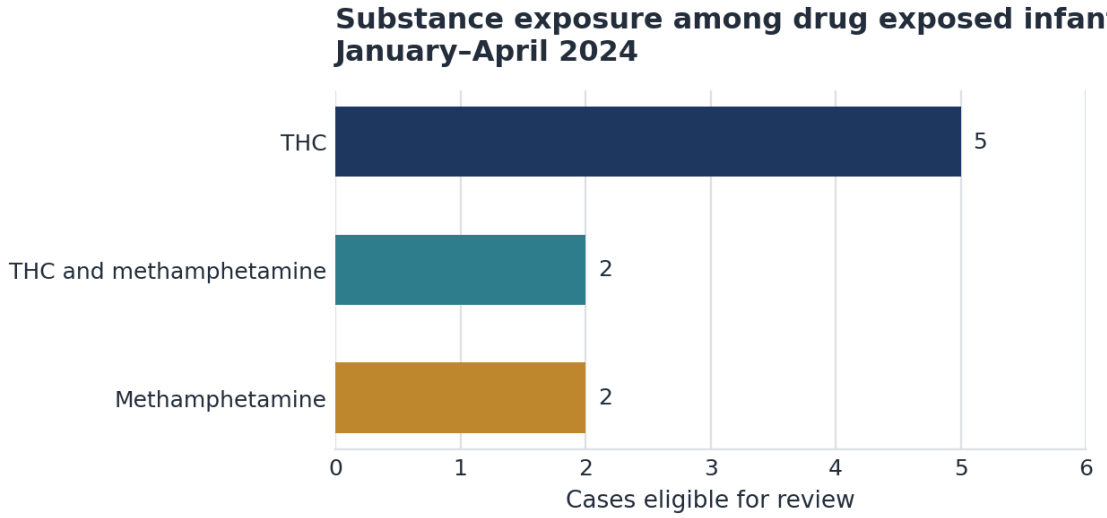


Figure 10. Substance exposure among drug exposed infant reviews, January–April 2024. Adapted from the 2024 Safe Systems reviews.[25]

Risk factors identified through quality assurance reviews

Risk factors from Safe Systems reviews are similar to those identified in Child and Family Services Reviews and Practice Model Reviews in 2023; both review types show substance use and economic instability occurring in more than 50% of cases. In the United States, Child and Family Services Reviews are conducted by child welfare agencies to ensure conformity with federal regulations and to gauge the experiences of citizens receiving state child welfare services.[26] Similarly, Indiana practice model reviews are peer review processes that use interviews with families and children, along with case record review, to identify system strengths as well as areas needing improvement.[27]

Case studies of critical incidents

WITHHELD FOR CONFIDENTIALITY

This section of the original memorandum presented two case studies of critical incidents reviewed by the Safe Systems team, drawn from confidential child fatality review material. Both narratives have been removed in full from this version. They contained detailed circumstances of individual child deaths — family composition, medical history, substance use, agency contact history and assessment findings — that, although not name-bearing, would be re-identifiable and are protected as confidential child abuse and neglect information under Indiana Code 31-33-18. The aggregate risk-factor findings that the case studies illustrated are presented in the Safe Systems section above.

Conclusion

Parental substance use disorder significantly affects the lives of children in both the short and the long term. The immediate effects are often manifested in the form of neglect, abuse and unstable home environments, leading to emotional, behavioral and academic difficulties. In the long run these children are at higher risk of developing substance use disorders themselves, perpetuating a cycle of addiction and its associated hardships.

These outcomes are not, however, inevitable. With early intervention, robust social support systems and therapeutic resources, children can overcome these challenges and lead healthy, productive lives. Further research is needed to develop effective strategies for supporting these children and breaking the cycle of addiction. This memorandum underscores the urgent need for attention to this vulnerable population in order to mitigate the effects of parental substance use disorder.

Notes

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2. Substance Abuse and Mental Health Services Administration et al., “Key substance use and mental health indicators in the United States: Results from the 2022 National Survey on Drug Use and Health.”
3. Ibid.
4. Indiana Department of Health, “Marijuana and tobacco use in Indiana.”
5. Prevention Insights, “2022 Indiana Youth Survey summary report.”
6. Slater and Alpert, “Apparent per capita alcohol consumption: National, state, and regional trends, 1977–2021.”
7. Patrick et al., “Alcohol use and the COVID-19 pandemic.”
8. “Prenatal opioid and substance exposure.”
9. Perez et al., “Prenatal methamphetamine — impact on the mother and child — a review.”
10. Welford, Danielsson, and Manhica, “Parental substance use disorder and offspring not in education, employment or training.”
11. Frederiksen, Hesse, and Pedersen, “Problematic parental substance use, childhood family structures and adverse outcomes in young adulthood.”
12. Frederiksen et al., “The impact of parental substance use disorder and other family-related problems on school related outcomes.”
13. Turner et al., “Children affected by household substance use or mental health issues.”
14. McGovern et al., “The association between adverse child health, psychological, educational and social outcomes, and nondependent parental substance.”
15. Hong et al., “Termination of parental rights for parents with substance use disorder.”
16. Khemiri et al., “Association of parental substance use disorder with offspring cognition.”
17. Raitasalo and Holmila, “Parental substance abuse and risks to children's safety, health and psychological development.”
18. Ibid.
19. Donohue et al., “Examination of the extent to which employment factors are associated with reduced child maltreatment potential and drug use.”
20. Kepple, “The complex nature of parental substance use.”
21. McNeely et al., “Performance of the Tobacco, Alcohol, Prescription medication, and other Substance use (TAPS) tool for substance use screening in primary care patients.”
22. Dawe et al., “Moderators and mediators of outcomes of parents with substance use problems.”
23. Safe Systems team, inquiry regarding substance use cases and Safe Systems, 9 May 2024. Respondent identity withheld.
24. Ibid.
25. Ibid.
26. Child Welfare Information Gateway, “Child and Family Services Reviews.”

27. “Practice Model Review,” DCS child welfare manual, chapter 2, section 16.

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